RCCOF Prompt Framework—INC-00531

Microsoft Copilot framework for summarizing external research papers | Internal Medicine Research

SCOPE STATEMENT

This framework is designed only for summarizing non-PHI, publicly available external research papers: journal articles, conference proceedings, and clinical practice guidelines. Do not use it with patient information, internal drafts, unpublished data, or any other institutional document.

The framework has five components, used in order: Role, Context, Constraints, Output format, Follow-up verification. Text in shaded boxes is the prompt language to paste into Copilot; replace anything in [square brackets] before sending. The framework reduces the risk of inconsistent or leaked content when the human verification steps are followed; it does not guarantee that no leakage will occur.

Before you run the prompt

1. Close every other document, browser tab, email, and chat that is not the paper you are summarizing.
2. Start a new Copilot chat. Do not continue a session that was used for any other work.
3. Attach or paste only the published paper. Confirm it is publicly available and contains no patient information.

1. Role

Defines what Copilot is acting as, and what it is not.

You are a clinical research summarization assistant supporting an internal medicine research team. Your only task is to produce an accurate, neutral summary of one published external research paper for an internal review meeting. You are not acting as a clinical advisor, a peer reviewer, or a strategy advisor.

2. Context

Identifies the source material and states what it is not.

The source material is one publicly available external paper: [title], by [authors], published in [journal, conference, or guideline body], [year], [DOI or URL], provided as [the attached file / the text pasted below]. It is a [journal article / conference proceeding / clinical practice guideline].

It is not an internal document, a draft, unpublished data, patient information, or institutional material. Treat this paper as the only source for this task. The readers are research staff and residents who will discuss the paper at an internal review meeting.

3. Constraints

Names what must be excluded. The first three are the privacy guardrails.

1. **No cross-document content.** Use only the paper identified in Context. Do not use, quote, or draw on any other open document, browser tab, file, email, chat, meeting, or earlier content in this session.
2. **No editorial commentary.** Do not add your own opinions, interpretation, critique, comparisons with other studies, or background knowledge that is not stated in the paper. Report what the authors state, including the limitations they state.
3. **No institutional strategy content.** Do not include or infer anything about this institution's plans, priorities, projects, funding, drafts, or internal discussions, even if such content is available in this session.
4. **No clinical recommendations of your own.** Report a recommendation only where the paper makes it, attribute it to the authors, and give its location in the paper.
5. **No gap-filling.** If the paper does not state something, write "Not stated in the paper." Do not estimate, invent figures, or invent citations.
6. **Flag anything you cannot confirm.** If any part of your answer may have come from a source other than the cited paper, say so in the Source check line.

4. Output format

Fixes the structure so every summary is comparable, with a citation requirement and verification as the required final action.

Return the summary under exactly these headings, in this order, in no more than 400 words, with no introduction or closing remarks:

1. **Citation:** authors, title, journal/conference/guideline body, year, DOI or URL.
2. **Paper type and objective:** one to two sentences.
3. **Methods:** two to four bullets covering design, population, setting, and sample size.
4. **Key findings:** three to five bullets, each ending with its location in the paper in brackets, e.g. [p. 6, Table 2] or [Section 3.1].
5. **Limitations stated by the authors:** bullets, each with its location in the paper.
6. **Authors' conclusions and recommendations:** attributed to the authors, each with its location in the paper.
7. **Source check:** one line stating either "All content above is drawn only from the cited paper" or a warning that lists any content that may have come from elsewhere.
8. **Final line, exactly:** "HUMAN VERIFICATION REQUIRED BEFORE USE: complete the three Follow-up verification checks."

5. Follow-up verification

Required final action. Send the follow-up prompt, then a person completes all three checks with the paper open. Do not circulate the summary until all three pass.

Follow-up prompt: For each key finding and each recommendation in your summary, quote the sentence from the paper that supports it and give its page or section. Then list any statement in your summary that does not come from the cited paper.

Check	What the reviewer does	If it fails
1. Citation accuracy	Compare the Citation line with the paper's title page: authors, title, source, year, DOI or URL. Open the paper at the location given for every key finding and confirm the statement and every number match.	Correct the entry from the paper, or delete any statement that cannot be found in it.

Check	What the reviewer does	If it fails
2. Cross-document content	Read the Source check line and act on any warning. Read the whole summary for anything that is not in the paper: internal project names, draft wording, institutional plans, or material from another document or tab.	Discard the whole summary. Close all other documents, start a new Copilot chat, and run the prompt again. Tell Dr. Patel if it happens again.
3. Clinical recommendations	Confirm every recommendation appears in the paper, is attributed to the authors, and keeps the paper's wording and strength. Confirm Copilot has added no advice of its own.	Delete any recommendation that is not in the paper. The summary is discussion material, not clinical guidance.

Reviewer sign-off: record the reviewer's initials and the date on the summary once all three checks have passed.

This framework contains structure and placeholders only. It was not built or tested with any institutional document, and no internal content appears in it. Questions about organizational AI-use policy go to the Information Governance team.

Coaching Notes—INC-00531

For Dr. Patel's research staff and residents | How to use the RCCOF framework for external paper summaries

Component explanations

Component	What it does	Why it matters here
Role	Tells Copilot its one job: summarize a single paper.	A narrow role stops it drifting into advice or critique.
Context	Names the exact paper and says what it is not.	Copilot knows which source counts, so summaries are complete.
Constraints	Lists what to leave out: other open documents, Copilot's opinions, institutional content.	This is the main privacy guardrail.
Output format	Fixes the headings, length, and citation.	Summaries are consistent, and every finding traces to a page.
Follow-up verification	A person checks the summary against the paper before sharing.	Copilot can still be wrong; this is where errors are caught.

Worked examples

Illustrative paper types only; no real or internal documents are used.

1. **Clinical outcomes study.** Context names a published randomized trial of a heart-failure medication as a "journal article". Expect design and sample size under Methods, and table references on each finding. Check every number against the tables.
2. **Public health guideline.** Context names a published national adult-vaccination guideline as a "clinical practice guideline". Recommendations should appear only under the authors' heading, in the guideline's own wording and strength.
3. **Conference proceeding.** Context names a published abstract on sepsis screening. Abstracts are short, so several "Not stated in the paper" entries are correct. Do not ask Copilot to fill the gaps.

Why each verification check matters

1. **Citation accuracy** prevents invented references, misquoted findings, and wrong numbers reaching your review meeting.
2. **Cross-document content** prevents internal drafts or institutional material appearing in a summary, which is the problem that prompted this framework.
3. **Clinical recommendations** prevents Copilot's own advice being mistaken for the authors' recommendations or for clinical guidance.

Common pitfalls

- **Using the framework with non-eligible content.** It is not designed for patient information, internal drafts, or unpublished data. If the source is not a public paper, stop.
- **Skipping verification under time pressure.** The three checks take a few minutes. An unverified summary should not be circulated.
- **Ignoring cross-document content warnings.** If the Source check line warns, discard the summary and rerun in a clean session. Deleting the suspect sentence is not enough.
- **Reusing an old chat or leaving other tabs open.** Close everything else and start a new chat each time.
- **Treating the framework as a guarantee.** It reduces risk only when the checks are completed.